

# Continent Cutaneous Diversion (Indiana Pouch)

WARWIKI

Patient Information · An inside pouch you empty with a catheter, with no bag

A continent cutaneous diversion is a pouch made from your own bowel. It stores urine inside your body. A small opening on your belly (a stoma) stays closed between uses. You do not wear a bag. Instead, you empty the pouch several times a day by passing a thin tube (a catheter) through the opening. The Indiana pouch is one well-known type.

## About This Procedure

A piece of **large and small bowel** is reshaped into a soft **pouch** inside the belly. The ureters (the tubes from the kidneys) drain into it. A narrow channel with a **one-way valve** is brought to the skin as a small, flat stoma. It is sometimes placed in the belly button.

- You pass a catheter to drain the pouch on a **schedule** your team sets.
- You **rinse** the pouch to clear mucus. The bowel keeps making mucus for life.
- **No bag.** A small pad or bandage covers the stoma.

It suits people who want no bag, cannot have or do not want a neobladder, and can **catheterize for life**. That takes steady hands and good eyesight, or a helper.

## Benefits and Risks in Numbers

about  
**9 in 10**

stay dry between  
catheterizations

about  
**15 in 100**

get a narrowed  
stoma that can  
make the catheter  
hard to pass

about  
**1 in 2**

need another  
procedure over the  
years; many are  
small

Numbers are averages from published studies. Your own risk may differ; ask your team.

## Learn the Terms

### Continent diversion

A urine system that does not leak. You choose when it empties.

### Indiana pouch

A continent pouch built from the right side of the large bowel and a bit of small bowel.

### Pouch

The inside sac, made from bowel, that holds urine.

### Stoma

The small skin opening you pass the catheter through.

### Catheterize

To pass a thin tube through the stoma to drain the pouch.

### Irrigate

To rinse the pouch with salt water to clear mucus.

### Ureters

The tubes that carry urine from the kidneys to the pouch.

### Mucus

Slippery fluid the bowel makes. Clearing it is part of daily care.

**Will it hurt?** You are asleep for the surgery, so you feel nothing. Afterward, your belly will be sore, and you get pain medicine. The bowel takes a few days to start working again. You wake up with a tube draining the pouch and thin stents in the ureters.

#### In the hospital

Walk early. Eat again slowly. Learn to rinse the pouch tube.

#### First weeks at home

The tube stays until the pouch heals. You may give yourself shots to prevent clots for about 4 weeks.

#### Tube out

Start catheterizing, often at first, then less often as your team advises.

#### For life

Catheterize, rinse, and keep your check-ups.

## How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop. Your team will tell you if you need a bowel prep.
- Your team plans the **stoma spot** with you and makes sure you, or a helper, can **catheterize for life**.
- Stop smoking**, and arrange help at home.

#### Tell your team ahead of time if you:

- Take a blood thinner, or have kidney or liver problems
- Have bowel disease, or trouble with your hands or eyesight

## What Happens During Surgery

- You are asleep and get antibiotics.
- A piece of bowel is set aside. The rest of the bowel is joined back together.
- The bowel is shaped into a pouch. A narrow channel with a valve is brought to the skin.
- The ureters are joined to the pouch. A tube and stents stay in while it heals.

**Your schedule:** many people end up emptying about every 4–6 hours in the day and once at night. Yours depends on how much the pouch holds. **For life:** blood tests (kidney function, acid level, vitamin B12) and kidney scans.

#### Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A catheter that is getting harder to pass, or new leaking from the stoma
- Redness, swelling or fluid leaking from the cut

#### Go to an emergency department if:

- You cannot pass the catheter, or no urine comes out. A full pouch must be drained soon.
- Bad belly pain, vomiting, or no gas or stool
- Chest pain, trouble breathing, or heavy bleeding
- You cannot reach the office and are worried

#### Three Things to Remember

- The pouch stores urine inside you. You empty it with a catheter, with no bag.
- Never skip or stretch out your emptying schedule. Rinse out mucus as shown.
- Keep your check-ups for life, even when you feel well.